

FY2027 Weight-Shift Procedure Concepts — Explained in Simple Terms

Concepts C37–C47 | Based on the FY2027 final MS-DRG Table 5 weights (CMS-1849-F, July 31 2026) and the 752-DRG year-over-year comparison | Each card: what it means, what we check, the guideline, and why it pays now

The simple idea behind this whole set: when CMS changes what a DRG pays, it changes what a coding mistake is worth. If a DRG's weight goes UP 36%, then every wrong code that pushes a claim INTO that DRG is worth 36% more than last year — same mistake, bigger overpayment. And when two sibling DRGs split apart (one up 30%, its twin down to the -10% cap), the single code that decides which sibling a claim lands in becomes the most valuable character in the grouper. These eleven concepts each guard one of those doors. Effective date matters: everything here applies to discharges on or after October 1, 2026, grouped on version 44.

The Winner DRGs — Codes That Push Claims IN Are Worth More

C37 Debridement / Skin Graft Family Sweep — DRG 465 up 36.1%	
In simple words	DRG 465 (wound cleaning surgery or skin graft) got the biggest raise in the whole table. To get into this DRG, a claim needs just one door-opening code: an excisional debridement (surgeon CUT dead tissue out) or a skin graft. Our oldest, highest-volume findings — 'the note says washout, not excision' — live exactly on this door. Same finding as last year, now worth about a third more. Bonus: the new FY2027 grouper also re-routes hip/knee removal-and-replacement code pairs into this family, so a brand-new population is flowing through the same door.
What the auditor checks	Every FY2027 claim in 463–465: which code opened the door (debridement, graft, or the new revision pair)? Does the operative note say tissue was cut away — the word excision or equivalent — or only washed, scraped, curetted? Is it the only OR code on the claim?
Guidelines referred	PCS root operation definitions (Excision vs Extraction); Convention A11 (coders cannot assume); A8 (every character must be supported); Coding Clinic 1Q 2017 (excisional debridement documentation); CMS-1849-F v44 grouping change.
Why it pays in FY2027	+36.1% on the family weight means every debridement/graft finding recovers ~36% more than the identical FY2026 finding. Three existing concepts (C01, C17, C33) stack on one claim population.

C38 The Burn Graft Gate — DRG 927 up 30.4% vs DRG 933 down 38.5% before cap	
In simple words	Two burn DRGs hold the SAME patients: extensive burns on a ventilator over 96 hours. The only difference: 927 has a skin graft code, 933 does not. This year 927 got a 30% raise while 933 took the maximum allowed cut. So one skin-graft code now decides a payment gap of roughly 40 percentage points — the single most valuable code in the FY2027 table. If a temporary covering (donor skin, pig skin, artificial skin sheet) or a dressing was coded as a real graft, the claim sits in the wrong sibling.
What the auditor checks	Every DRG 927 claim, two doors: (1) GRAFT — does the note show a true graft (root operation Replacement) with a donor site for autografts, or only a temporary covering applied? (2) VENTILATOR — do the vent hours truly exceed 96 (our C11 recount)? Fail either door and the claim regroup. Also watch providers whose graft-coding rate on burn claims suddenly jumps after October 1.
Guidelines referred	PCS Replacement definition; device characters (autologous 7 vs nonautologous K vs synthetic J); B6.1 (what remains on/in the patient); Coding Clinic on temporary burn coverings; Coding Clinic 4Q 2014 (counting ventilator hours).
Why it pays in FY2027	Widest sibling divergence in FY2027; small population, so 100% review is affordable and every hit swings the full 927-vs-933 differential.

C39 Knee Procedures — DRG 488 up 25.5% next to the new infection DRG 400	
In simple words	Knee surgery WITHOUT an infection diagnosis (488) got a 25% raise, in the same year CMS built a separate, different-paying DRG (400) for knee surgery WITH an infection as the main diagnosis. So the routing question — 'was the admission really for the mechanical knee problem, or for the infection?' — now moves real money, and so does the surgery code itself (a scope washout coded as open surgery).
What the auditor checks	FY2027 claims in 488/489: is the knee procedure supported by an operative report? Is the approach truly open, or was it arthroscopic? Is an infection documented in the record but coded as a secondary diagnosis so the claim reaches the up-weighted no-infection DRG?
Guidelines referred	PCS B4.1a (right body part) and approach definitions; A8; UHDDS principal diagnosis definition; CMS-1849-F (creation of DRG 400 and 403/404, deletion of 485–487).
Why it pays in FY2027	+25.5% on 488 raises the reward for both wrong-approach coding and infection-PDX steering; year one of a new sibling DRG is when routing errors peak.

C40 Liver / Pancreas 'Other Procedure' DRG 425 up 25.4%	
In simple words	DRG 425 is a leftover bucket: liver, gallbladder, and pancreas operations that don't fit any specific DRG land here. Leftover buckets are where coding mistakes pool — a gallbladder surgery with a wrong approach character, or a drain placement wrongly coded as open surgery, falls into this bucket instead of its correct (usually cheaper) home. The bucket just got a 25% raise, so the pooled mistakes pay more.
What the auditor checks	Every FY2027 claim in 423–425, short stays first: does the 0F procedure code's approach and body part match the note? Would the corrected code route the claim to its specific family (e.g., laparoscopic cholecystectomy DRGs) or collapse it to a medical DRG?
Guidelines referred	PCS approach definitions (B5.2, B5.3); B4.1a; A8; MS-DRG Definitions Manual residual-routing logic.
Why it pays in FY2027	Residual DRGs have the highest miscode density per claim; +25.4% makes 423–425 the best-paying place to find routing errors.

The Loser DRGs — Watch Codes That Pull Claims OUT

C41 Loser-DRG Exit Monitor — five DRGs cut to the -10% cap	
In simple words	Five DRGs took the biggest cuts, all limited to -10% this year — with the REST of the cut arriving next year. When a DRG starts paying less, some providers' coding drifts toward the better-paying sibling: burn claims grow graft codes (933 to 927), transplant claims grow CC/MCC capture (017 to 016). The monitor doesn't accuse anyone — it watches volumes quarter by quarter and routes suspicious shifts to the gate-validation concepts.
What the auditor checks	Per provider: volume in each capped loser DRG (819, 017, 933, 734, 817) versus volume in its natural destination sibling, FY2026 vs FY2027. A trough in the loser plus a spike in the sibling sends the sibling claims to the gate check (graft code, qualifier, tier).
Guidelines referred	CMS 10% cap policy in CMS-1849-F (the cap applies only when a DRG keeps its number, and defers — not cancels — the rest of the cut); the gate definitions of each sibling pair.
Why it pays in FY2027	Migration incentive compounds into FY2028 as the deferred cut lands; catching the drift in Q1–Q2 sets the baseline for extrapolation later.

Additional Gates (C42–C47)

C42 Burn Graft Gate, Whole Family — DRGs 928/929 vs 934/935	
In simple words	The same graft-or-no-graft split from C38 runs through the rest of the burn chapter: full-thickness burns with a graft (928/929) versus without (934/935). Everything said about 927/933 applies here — a temporary covering coded as a graft parks the claim in the wrong, better-paying sibling.
What the auditor checks	All FY2027 with-graft burn claims: true Replacement documented, donor site for autografts, or only a covering/dressing applied?
Guidelines referred	PCS Replacement definition; device characters; B6.1; Coding Clinic on burn wound coverings.
Why it pays in FY2027	Extends the highest-leverage single-code gate across the entire burn MDC instead of one DRG pair.

C43 Bone Marrow Transplant — Self vs Donor Qualifier (DRG 017 cut 39.2% before cap)	
In simple words	Autologous transplant means the patient's OWN stem cells; allogeneic means a DONOR's. The transplant procedure code carries one character that says which — and the donor version groups to a much higher-paying DRG family. With the autologous DRG's big cut, that one character is now a family-jump lever: self cells coded as donor cells moves the whole claim up.
What the auditor checks	FY2027 claims in the transplant DRGs (014/016/017): does the transfusion code's qualifier match the transplant note — was there actually a donor? Allogeneic-coded claims must show donor documentation.
Guidelines referred	PCS qualifier definitions for stem-cell/bone-marrow transfusion codes (autologous vs nonautologous); A8; A11.
Why it pays in FY2027	One character = one DRG family; the FY2027 cut to 017 sharpens the steering incentive toward the donor family and toward CC/MCC capture (which the D-series watches).

C44 Radical vs Total Hysterectomy — the New 731–733 Malignancy Family	
In simple words	FY2027 rebuilt the uterine-cancer surgery DRGs (old 736–741 became new 731–733), while the radical-surgery DRG 734 took a 25% pre-cap cut. 'Radical' hysterectomy is a bigger operation than 'total' — it removes tissue beyond the uterus. That extra extent must be in the operative note; a total hysterectomy coded as radical overstates the operation. And because 731–733 are brand new, year-one routing mistakes (malignancy diagnosis missing, wrong procedure combination) have no historical pattern to catch them — except this audit.
What the auditor checks	FY2027 claims in 731–735: does the note document the radical extent (parametrium, upper vagina) or only the uterus/cervix removal? Is a malignancy diagnosis actually present on 731–733 claims?
Guidelines referred	PCS body-part definitions for the female reproductive chapter; B4.1a; A8; UHDDS; CMS-1849-F (736–741 deletion, 731–733 creation).
Why it pays in FY2027	New family + cut sibling = both routing-error risk and extent-overstatement risk concentrated in one small, reviewable population.

C45 Antepartum OR-Procedure Gate — DRGs 817–819 (largest pre-cap cuts)	
In simple words	These DRGs hold pregnant patients (before delivery) who had an operating-room procedure — that OR code is the only thing separating them from the cheaper medical antepartum DRGs. Our standard checks apply with full force here: is there an operative report at all (C03)? Was the procedure completed or aborted (C04)? An unsupported OR code collapses the claim to medical. Important honesty note: the DRG's cut does not make a false code acceptable — a loser DRG still overpays when the gate code is wrong.
What the auditor checks	Short-stay FY2027 claims in 817–819: operative report exists in this stay, matches the billed date, and the procedure was completed as coded.
Guidelines referred	Federal Register 31-Jul-1985 / UHDDS substantiation; PCS A8; B3.3 (discontinued procedures).
Why it pays in FY2027	Surgical-to-medical collapse remains the full differential regardless of the weight cut; short antepartum stays with OR codes are a compact, high-hit population.

C46 Burn-Wound Excisional Debridement — the Burns Version of C01	
In simple words	Burn care has its own version of the oldest finding in the book: the note says the burn was cleaned, scrubbed, or dressed — but the code says dead tissue was surgically EXCISED. In the burn chapter that excision code contributes to which sibling the claim reaches, alongside the graft gate.
What the auditor checks	FY2027 burn-family claims carrying a 0HB/0JB excision code: does the note say excised / cut away / tangential excision — or only cleansed, scrubbed, non-excisional debridement, dressing change?
Guidelines referred	PCS Excision vs Extraction definitions; A11; Coding Clinic on burn debridement documentation.
Why it pays in FY2027	The C01 language test transfers verbatim onto the burn population, whose sibling weights just diverged.

C47 Autograft Without a Harvest — the Missing Donor Site	
In simple words	A real autograft (device character 7 — the patient's own skin) has to come FROM somewhere: a donor site, harvested in the same operation. A claim coding an autograft with no harvest code and no donor-site documentation is telling an incomplete story — usually the truth is a temporary covering or a skin substitute, which codes differently and often groups differently. This is a mechanical, claims-visible contradiction: the money character (autologous) with its supporting code absent.
What the auditor checks	Any FY2027 OHR graft with device character 7 in the up-weighted families: is there a harvest/donor-site excision code on the claim, and a donor site in the note? If not: device-character swap or root-operation downgrade.
Guidelines referred	PCS B3.9 (autograft harvested from a different procedure site is coded separately); device key definitions; B6.1; A8.
Why it pays in FY2027	Runs as a cheap structural screen INSIDE the biggest winner families (463–465, burn DRGs) — a self-contradicting claim needs no clinical judgment to select.

How To Read This Set

Run order: start where one code decides everything and populations are small (C38, C42, C47), move to the volume engine (C37, with C46 layered on the burn side), then the family-jump gates (C43, C44), then routing validation (C40, C39, C45), with C41 running permanently each quarter as the drift detector. One caution repeated from the workbook: FY2027 dollar values stay provisional until CMS releases the FY2027 Pricer, and every valuation must regroup the claim on grouper v44 for discharges on or after October 1, 2026 — v43 or earlier for older claims. Weight-shift percentages cited are the Bright Spot Insights Table-5 comparison of the 752 DRGs present in both years.